SOCIAL FINANCE

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Standards Development Working Group

26th March 2026

PRIVATE AND CONFIDENTIAL

Agenda

14:30 -14:40 (10m)	**Welcome and housekeeping** Andrew Newman, ODI
14:40 -14:45 (5m)	**Joined Up Care - Adult Social Care Update** Juliette Clewlow, Social Finance
14:45 – 15:25 (40m)	**Data Model Deep Dive (Interactive Session)** Andrew Newman, ODI
15:25 – 15:50 (35m)	**Data Model Prioritisation (Interactive Session)** Stephen Rice, Harry Angus, and Charlotte Moberly, Social Finance
15:50 -16:00 (10m)	**Next Steps**

Things to be aware of

Conflicts of interest – Please inform the chair / secretariat if you think there is a risk.

Anti-competitive activities – Please inform the chair / secretariat if you think there is a risk of anti-competitive activity.

Intellectual Property - If you are concerned about the use of your contributions, please contact the secretariat.

Meeting Recording - We will record the meeting and using AI transcription tools.

Working together

1. Every voice matters
2. Please join in on the conversation, we don’t want this to be one way
3. If you’re happy to, keep your camera on
4. Please raise your hand if you want to come in, or drop your ideas in the chat
5. Respect the diverse viewpoints in the room, assume positive intent and remain open for different experiences
6. We will be using Miro to support our discussions today

News!

Receive our regular newsletter

Sign up at:
- http://eepurl.com/jnmjYs

Website: https://socialcaredata.github.io/ (start here!)

GitHub: https://github.com/SocialCareData

We are using GitHub to work openly:

- Store our specifications
- Track feedback on specifications (Issues)
- Openly share project documentation
- Have Discussions to inform the development of standards

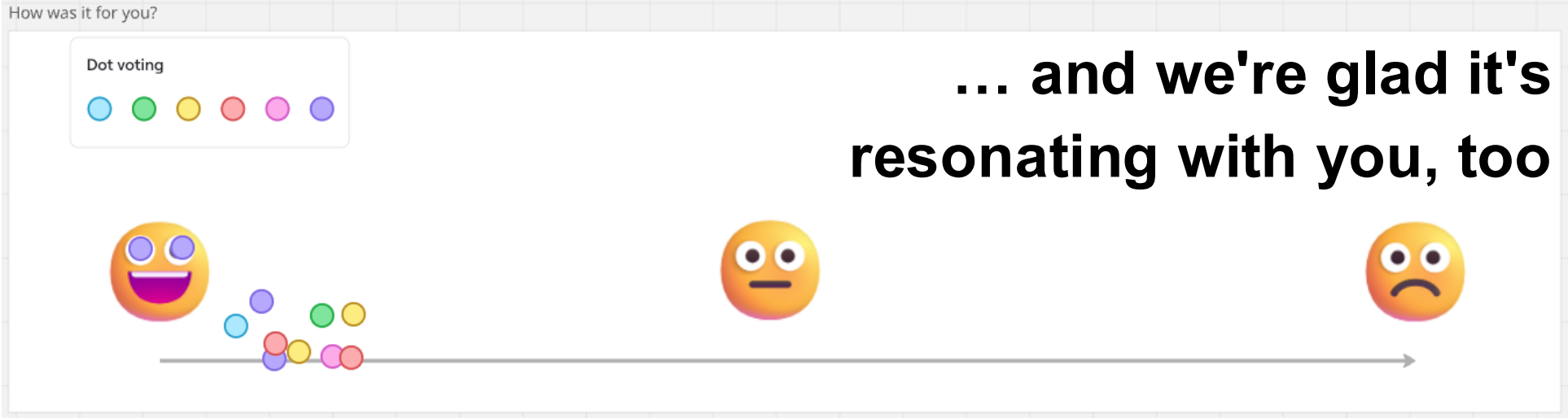
- In the future we will use GitHub to:
- Host and manage the development of any infrastructure or tools we need to develop
- Enable people to reuse our specifications and code

We will work to keep this simple and provide guidance when needed.

Thanks for your contributions so far!

This format is working for us...

- Your expertise is helping us:
  - Understand nuances in Child and Adult Social Care, and the data that underpins them
  - Turn ideas for standards into practical drafts
  - De-scope and pull back on where we're too ambitious



Since the last working group

You told us:

- Risk is too subjective and nebulous of a concept for us to include in our data standards
- We had massively overlooked the distinction between an organisation and a service
- For clarity, we should use controlled vocabularies rather than free-text fields to describe professional involvements and personal relationships

What we did:

- Removed all mention of risk
- We added a specification to describe an organisation, with organisations being linked to one or more services and services administered by one or more organisations
- We sought terminology lists for us to re-use for involvements and relationships. We'll show you, later!

You also told us about the importance of statuses

- With "statuses" being atomic attributes or properties of a person, eg. NEET, disabled
- But this has been difficult to square.
- We'll come back to this...

Joined up care (ASC)

The opportunity to improve joined up care

Our priority use cases:

Multi-agency enquiries for assessment and review

- Easily accessing information to complete a **care needs assessment** e.g. *GP/ mental health*
- Easily accessing information to **review a care plan** e.g. from *care providers*

Timely notifications and updates to social care

- Ensuring social workers have timely information about **hospital admissions and discharge**
- Social workers see **updates about support** provided to a person e.g. from *mental health or reablement*

Transitions from CSC to ASC

- Adult social workers can receive, track and act on information about young people **transitioning from children's to adult's services** e.g. from *CSC CMS record*

Recording and sharing assessments and plans

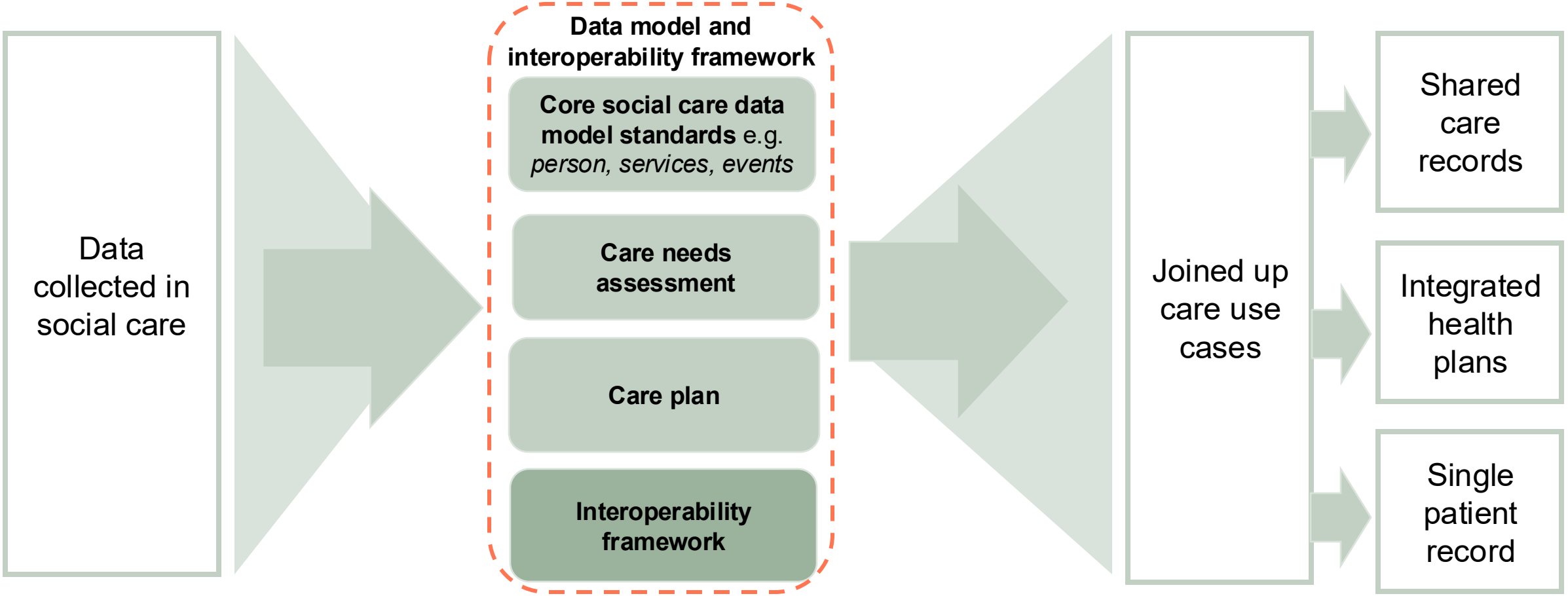
- **Recording assessments** easily into the CMS (incl from transcription tools)
- **Sharing care needs assessments** and **care plans** more easily with partners (and for analysis and reporting)

We will develop data model standards that support these joined-up care use cases

Our proposed standards and which use cases they support

Standards to develop	Multi-agency enquiries for assessment and review	Timely notifications and updates to social care	Transitions from CSC to ASC	Recording and sharing assessments and plans
Core social care data model standards e.g. *person, services, events*	✓	✓	✓	✓
Care needs assessment	✓		✓	✓
Care plan	✓			✓

We want to build a foundation for interoperability in social care that supports future strategic initiatives

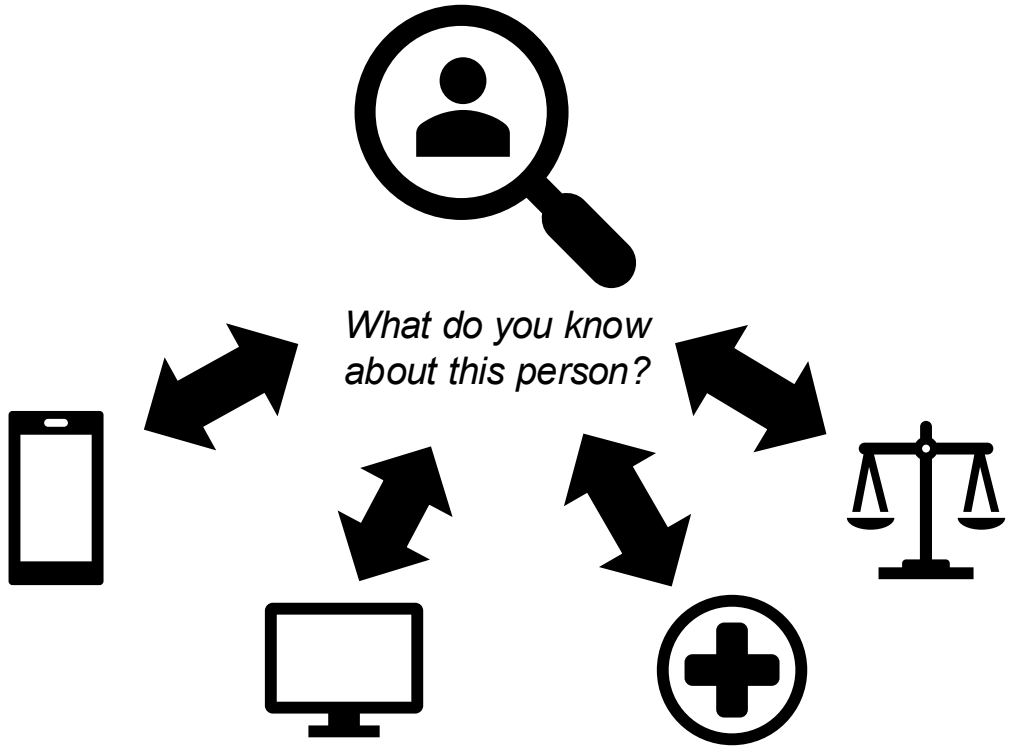


Any questions?

Data Model Deep Dive

Introduction

- We're working on better data infrastructure for multi-agency information sharing (MAIS) in social care
- This encompasses understanding which agencies across the country may know about a person and what they might know about them
- A more complete set of information means better decisions, better care, and better safeguarding + wellbeing outcomes.



Current state of play: What does our data model describe?

Person standard

Services standard

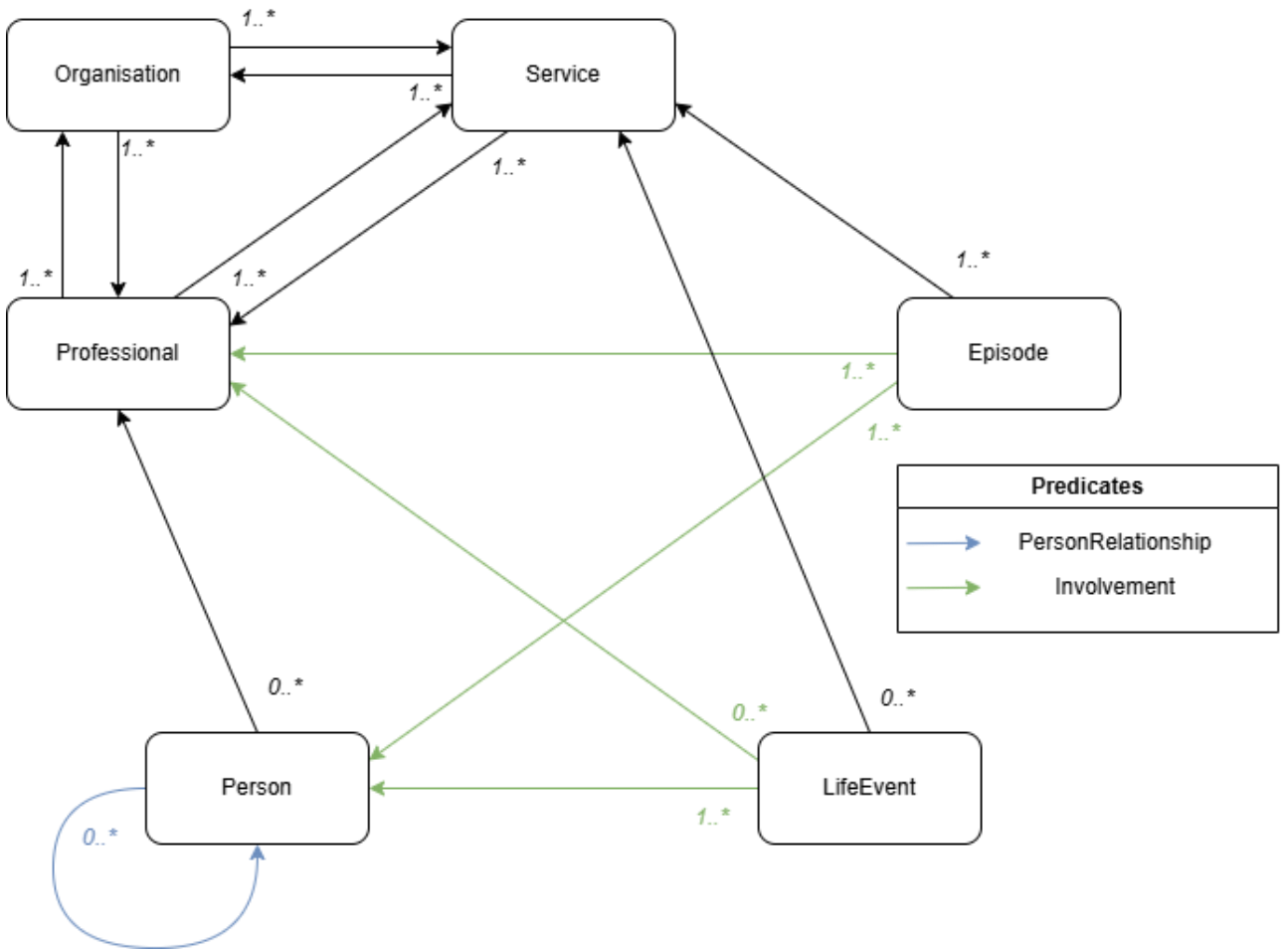
Professional standard

Relationship standard

Organisation standard

Life Event standard

Service Episode standard



Today

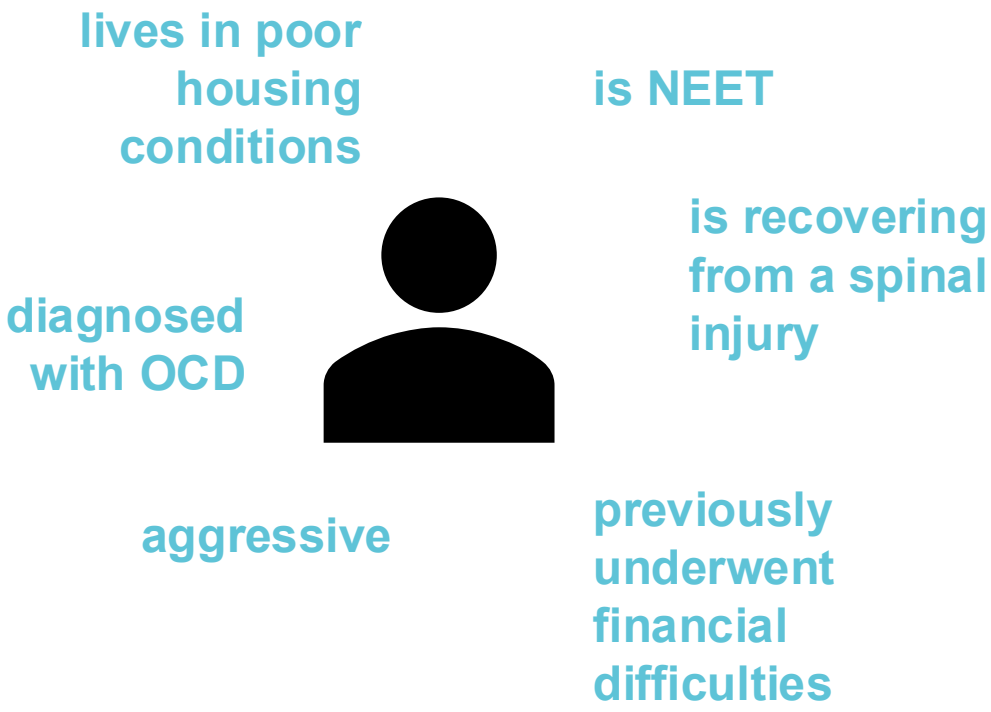
- In the last working group meeting, you all contributed to the massive effort of validating our approach and our draft specs
- 3 problems we can't wrap our heads around ourselves.
  - Statuses
  - Connected People
  - Adult Social Care modifications
- Today will be a much more discussion-oriented session, where you can tell us your viewpoints on each.

Challenge 1: Statuses

- You previously told us how important "statuses" are in social care, these being bite-sized attributes of a person that describe risk factors, vulnerabilities, or other things a social worker needs to know.

- This hasn't *really* been addressed by other data standards apart from SNOMED which allows documented "findings" about a patient.

- So we're operating with a relatively blank slate.

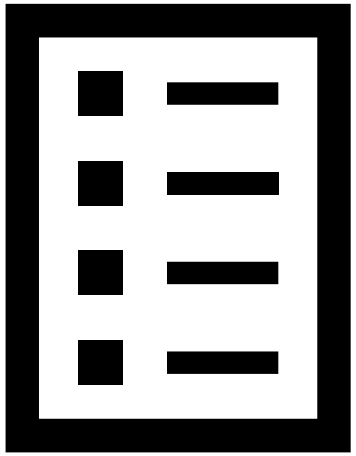


Challenge 1: Statuses | What would we do?

Extend the person spec to enable linked "statuses" for a person record, with some extra important fields

How we describe a status	
Status.attribute	The status, eg. "NEET"
Status.startDate	The date the person first assumed this status
Status.endDate	The date this status was observed to have ended

Build a vocabulary of terms that these statuses should conform to



Note: this list can start with TfD and Family Context, or can instead be built from a SNOMED Social Determinants of Health extension

Challenge 1: Statuses | What's the problem?

- This isn't really how things work right now. Social workers, health workers, police write their notes about a person as case notes as part of a service episode or life event.
- In fact, this also means that there is ambiguity and duplication between episodes/events and statuses.

  If a family has a successful application for free school meals, that's recorded as a service episode; would the family's social worker also have to record it as a status?

- We'd need to ensure statuses are audited often so that outdated information isn't being served
- And operationally, it's not easy to make the list and keep it updated. And a big list is harder for frontline workers to use

Obviously there are massive benefits, but the above problems stopped us from moving forward. What do you think?

Challenge 1: Statuses | How could we tackle this?

Forge ahead

- The benefits statuses could bring are enormous, given they are concise summaries of information that can massively change the way a person's care is approached

Make statuses linked with episodes/events

- The two problems – frictions with ways of working, and data duplication – are both the result of keeping statuses separate from episodes and events
- But as we mentioned previously, they're interlinked. A status "temporarily impaired movement" is downstream of a health service episode like a hospital stay
- So maybe there's a way to "derive" statuses from episodes and events?

Postpone or de-scope

- Maybe statuses aren't so important after all?

Challenge 2: Connected People

- We asked you a question last month that you felt you couldn't answer, so we're here with more information
- The current data model assumes that a person that is connected to another person *(eg. their father, their sibling, their friend)* is recorded as such via the **Person** specification
- That is, both people are recorded as **Person**s
- But the **Person** spec is pretty strict!

What our Person spec *requires*	
Identifier(s)	Name
Date of Birth	Address
Gender	

The problem is: would we have all this information for every connected person? Is it reasonable to expect *eg. the DoB for an enstranged father* to be recorded?

Challenge 2: Connected People | How could we tackle this?

Forge ahead

- Are we overthinking this?
- Maybe the only connected people that are recorded are those that are known to services, and if they're known to services, this sort of information is more likely to have been recorded.
- In other words, we're not trying to describe literally every relationship a person has.
- So just using the strict **Person** is fine.

Allow more flexibility in Person

- Maybe, as we look towards the metadata layer that could sit on top of a **Person** record, we could build in some flexibility
- DoB, address, etc. could all be estimated, provided some metadata is there to make that clear

Create "RelatedPerson"

- Instead of making the Person spec more flexible, we could just create a new object, a **RelatedPerson**.
- This would have fewer mandatory fields, with DoB, Address, etc. optional

The Problem

Each of these options introduces uncertainty and noise to the data. An estimated DoB or an unknown address will make ***matching*** difficult.

Enabling matching is the underlying principle of our work. If a social care record of a person's enstranged father can be matched to the father's criminal record held on the other side of the country, the social worker has vastly more information at their fingertips. But if the two records don't cleanly match – one has DoB and address, the other doesn't – the matching is less likely to happen, with potentially severe consequences.

Challenge 3: Making changes for Adult Social Care

- As we move towards a spec that encompasses ASC as well as CSC, we need to make some changes.
- There are statutory requirements for data collection about individuals (eg. via the Health and Social Care Act)
- These look fine to us, and we could just make the changes to the **Person** spec now, adding these elements.
- But is there nuance that we're missing? Is there a data minimisation problem?

ASC additions required for the Person spec	
Ethnicity (according to the NHS vocabulary)	Estimated age
Date of Death	Other CLD stuff like employment status or accommodation

Challenge 3: Making changes for Adult Social Care | Options

Change the Person spec

- Add Ethnicity and other data fields to the current Person spec
- Note that Ethnicity would have to not be mandatory for Children, but would be for Adults?

Split Person into Child and Adult

- Is it worth changing our approach of treating every individual as a Person to instead categorising them as Child or Adult?
- This would enable greater flexibility in the data fields of each
- But would probably cause issues with the CSC to ASC transition.

Something else

- Are we missing something here?

Something to think about: Service Episodes and Life Events

- Our data model currently allows the description of both service episodes and life events
- However, the specification for each one is almost exactly the same; the only difference is that service episodes have mandatory links to services/organisations/professionals

Are both necessary?

Let us know!

Thanks for your help!

Data Model Prioritisation

Can anyone tell me what this is?

Prioritising data items: What is essential for a social worker to know?

Scenario: You are a practitioner who has just had a case come into your tray. You're attempting to find out more information – what **key information** do you need from across the safeguarding system to make effective decisions?

We have grouped our data items into four categories:

Category	Definition	Example
Must Have	Essential for this to work at all. What data is essential to be able to make decisions and identify where to seek more information?	*"It's no good sharing me the fact that they are open to an education setting, every child is, I need to know where they are going to be able to find out more"* *"I need to know if they have a care plan in place"*
Should Have	What data would we like to see?	*"I'd like to know what other schools they've attended in the past 3 years"*
Could Have	What information is lower priority and could wait until a social worker engages another professional	*"Attendance might be useful context, but it's not a big part of my decision making and I can find out when I call the safeguarding lead"*
Would Have	What information would be helpful but is likely to be unfeasible?	*"I'd like to know more about their behaviour, but I know I'm not going to get that"*

Head over to Miro

https://miro.com/app/board/uXjVGuhtHLU=/?share_link_id=809566501355

Password: standards2603

Wrapping up

Next steps

- Frontline Engagement Session
- Vendor Engagement Session

Working Group Meetings:

- []

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Thank you.

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